

Clinical Effectiveness

METIMMOX-1 Economic Evaluation

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1 Overview

This report presents clinical effectiveness results from the METIMMOX-1 trial, evaluating biomarker-guided treatment strategies for selecting immunotherapy candidates in metastatic MSS/pMMR colorectal cancer.

The analysis uses **Firth-corrected Cox proportional hazards regression** with profile likelihood confidence intervals to assess treatment effect heterogeneity across three candidate biomarkers:

- **CRP** (C-reactive protein): Low CRP (<5 mg/L) at baseline
- **TLR** (Tumor Lesion Reduction): Early response (10% reduction at first CT scan)
- **TMB/BRAF**: High tumor mutational burden (9 mut/MB) or BRAF mutation

We compare three model structures to assess the robustness of treatment-biomarker interactions:

- **Model A**: All-in-one model with all biomarkers and interactions
- **Model B**: Focused models with all biomarkers but only one interaction per model
- **Model C**: Single-biomarker models with one biomarker and its interaction

2 Methodological Notes

2.1 Software Packages

This analysis uses the following R packages:

- **coxphf** version 1.13.4: Firth-corrected Cox regression with profile likelihood CIs
- **survival** version 3.8.3: Cox regression and Schoenfeld residual diagnostics
- **glmnet** version 4.1.10: Ridge regression sensitivity analysis
- **vcd** version 1.4.13: Cramér's V for biomarker correlation assessment

2.2 Firth-Corrected Cox Regression

We use Firth's penalized maximum likelihood method for Cox regression, which addresses:

1. **Monotone likelihood**: When a covariate perfectly separates events from non-events
2. **Small sample bias**: Reduces bias in coefficient estimates with sparse data
3. **Unstable confidence intervals**: Profile likelihood CIs are more reliable than Wald-based CIs in small samples

Statistical inference is based on **penalized likelihood ratio tests (PLRT)**, which are more appropriate than Wald tests for testing treatment-biomarker interactions in this setting.

2.3 Ridge Regression Sensitivity Analysis

We use Ridge regression as a sensitivity analysis to assess the stability of our interaction estimates. Ridge regression (also called L2-penalized regression or Tikhonov regularization) belongs to the family of regularized or penalized regression methods. In survival analysis, this is implemented as a penalized Cox model.

How Ridge regression works:

- Ridge adds a penalty term that discourages large coefficients. The bigger a coefficient tries to get, the more the model resists, pulling estimates toward zero (HR toward 1.0).
- Unlike LASSO ($\alpha = 1$), which sets some coefficients exactly to zero and performs variable selection, Ridge ($\alpha = 0$) shrinks all coefficients but retains all predictors.
- The degree of shrinkage is controlled by the tuning parameter λ , which is selected via **cross-validation (CV)**: the data are repeatedly split into training and validation sets, and the λ that produces the best predictive performance is chosen.

Why not LASSO or Elastic Net?

We determined that LASSO and elastic net are **not appropriate** for this analysis:

1. **Correlated biomarkers:** LASSO arbitrarily selects one of correlated predictors while setting others to zero, which does not reflect the clinical question of comparing ALL biomarker strategies.
2. **Hierarchy principle violation:** LASSO may select interaction terms (e.g., `crp:Rx`) while setting the main effect (`crp`) to zero, producing models that are difficult to interpret clinically.
3. **Clinical objective:** Our goal is to estimate treatment effect heterogeneity across ALL biomarkers simultaneously, not to select a “winning” biomarker.

Elastic net ($\alpha = 0.1\text{--}0.3$) represents a compromise but does not fully address these concerns.

Interpreting Ridge results:

Ridge regression is not replacing the Firth estimates—it provides a reality check. When Ridge substantially shrinks an interaction coefficient toward zero while Firth estimates a large effect, this indicates the effect is too unstable to trust for practical use. The CV-selected lambda reflects how much regularization is needed for reasonable out-of-sample prediction given the sample size and collinearity structure.

2.4 Multiple Testing Correction

We apply the **Benjamini-Hochberg (BH) procedure** to control the false discovery rate (FDR) on the primary Firth analysis:

- 6 interaction tests: 3 biomarkers $\times$ 2 outcomes (OS, PFS)
- Both raw PLRT p-values and BH-adjusted q-values are reported
- The Ridge sensitivity analysis does not require separate multiplicity correction

3 Data Preparation

3.1 Sample Characteristics

Table 1: Sample Characteristics

Characteristic	N (%)
Total patients (complete cases)	65
Control arm (FLOX alone)	29 (44.6%)
Experimental arm (alternating FLOX/nivolumab)	36 (55.4%)
Deaths (OS events)	56 (86.2%)
Progressions (PFS events)	48 (73.8%)
CRP-positive (low CRP)	22 (33.8%)
TLR-positive (early response)	41 (63.1%)
TMB/BRAF-positive	29 (44.6%)

3.2 Biomarker Correlations

Note: Cramér’s V interpretation: <0.1 = Negligible, $0.1\text{--}0.3$ = Weak, >0.3 = Moderate/Strong. The observed correlations are weak to negligible. While modest, any correlation among candidate biomarkers supports our decision not to use LASSO, which would arbitrarily select among correlated predictors.

Table 2: Biomarker Correlations (Cramér’s V)

Biomarker Pair	Cramér’s V	Interpretation
CRP vs TLR	0.278	Weak
CRP vs TMB/BRAF	0.077	Negligible
TLR vs TMB/BRAF	0.019	Negligible

4 Proportional Hazards Assumption

4.1 Schoenfeld Residual Tests

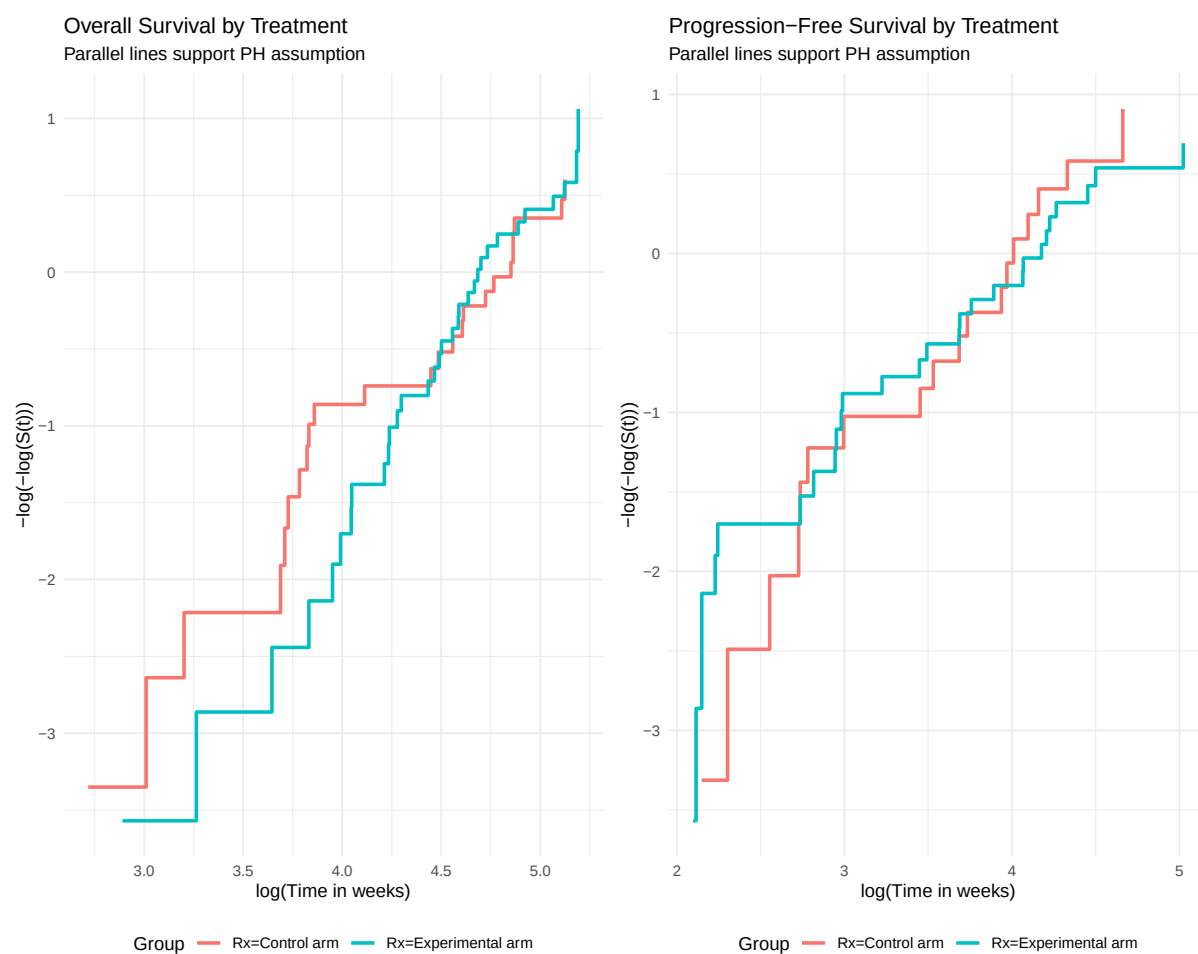
The proportional hazards assumption was tested using scaled Schoenfeld residuals. A significant p-value indicates potential violation of the PH assumption for that covariate.

Table 3: Schoenfeld Residual Test for Proportional Hazards Assumption

Covariate	² (OS)	p-value	² (PFS)	p-value
Age	2.84	0.092	0.42	0.515
sex	0.55	0.460	0.13	0.715
Rx	1.05	0.306	0.31	0.579
crp_num	0.37	0.543	1.36	0.243
tlr_num	0.19	0.665	1.00	0.318
tmb_braf_num	0.57	0.449	8.43	0.004
Rx:crp_num	1.65	0.199	0.35	0.554
Rx:tlr_num	1.98	0.159	1.60	0.206
Rx:tmb_braf_num	0.18	0.676	6.78	0.009
GLOBAL	7.38	0.598	24.23	0.004

Note: GLOBAL is the omnibus test for all covariates. Individual covariate tests help identify specific PH violations.

4.2 Log-Log Survival Plots



Interpretation: Approximately parallel lines on the log-log plot support the proportional hazards assumption. Crossing or converging lines suggest time-varying effects.

5 Primary Analysis: Firth-Corrected Cox Models

5.1 Model Type A: All-in-One Model

This model includes all three biomarkers as main effects and all three treatment-biomarker interactions simultaneously.

Formula: `Surv(time, event) ~ Age + sex + Rx + crp + tlr + tmb_braf + crp:Rx + tlr:Rx + tmb_braf:Rx`

5.1.1 Overall Survival (Model A)

Table 4: Model A Overall Survival - Full Coefficient Table

Term	HR (95% CI)	PLRT p-value	BH-adjusted p
Treatment × Biomarker Interactions			
RxExperimental arm:crp_num	0.71 (0.17-3.56)	1.000	1.000
RxExperimental arm:tlr_num	2.63 (0.75-8.74)	1.000	1.000
RxExperimental arm:tmb_braf_num	0.92 (0.29-2.91)	1.000	1.000
Main Effects and Covariates			
Age	1.01 (0.98-1.04)	—	—
sex1	1.48 (0.82-2.68)	—	—
RxExperimental arm	0.62 (0.23-1.77)	—	—
crp_num	0.70 (0.18-2.06)	—	—
tlr_num	0.20 (0.08-0.55)	—	—
tmb_braf_num	0.93 (0.40-2.12)	—	—

5.1.2 Progression-Free Survival (Model A)

Table 5: Model A Progression-Free Survival - Full Coefficient Table

Term	HR (95% CI)	PLRT p-value	BH-adjusted p
Treatment × Biomarker Interactions			
RxExperimental arm:crp_num	0.22 (0.04-1.59)	1.000	1.000
RxExperimental arm:tlr_num	0.87 (0.22-3.22)	1.000	1.000
RxExperimental arm:tmb_braf_num	0.48 (0.12-1.93)	1.000	1.000
Main Effects and Covariates			
Age	1.00 (0.97-1.03)	—	—
sex1	1.26 (0.64-2.52)	—	—
RxExperimental arm	1.88 (0.61-6.61)	—	—
crp_num	1.75 (0.31-7.00)	—	—
tlr_num	0.14 (0.05-0.45)	—	—
tmb_braf_num	0.80 (0.30-2.09)	—	—

5.2 Model B: Focused Models

These models include all three biomarkers as main effects but only ONE treatment-biomarker interaction per model. This approach tests each interaction while controlling for all biomarkers.

Table 6: Model B: Focused Models - Treatment $\times$ Biomarker Interaction Summary

Interaction Term	Overall Survival		Progression-Free Survival	
	HR (95% CI)	PLRT p	HR (95% CI)	PLRT p
CRP $\times$ Treatment	0.97 (0.26-4.54)	1.000	0.19 (0.03-1.34)	1.000
TLR $\times$ Treatment	2.47 (0.75-7.75)	1.000	0.75 (0.20-2.66)	1.000
TMB/BRAF $\times$ Treatment	0.89 (0.28-2.79)	1.000	0.45 (0.11-1.78)	1.000

Note: Each row represents a separate model with all biomarkers as main effects but only the specified interaction.

5.2.1 Full Coefficient Tables

Table 7: Model B CRP-Focused: Overall Survival - Full Coefficients

Term	HR (95% CI)	p-value
RxExperimental arm:crp_num	0.97 (0.26-4.54)	0.970
Age	1.01 (0.98-1.04)	0.553
sex1	1.53 (0.86-2.77)	0.148
RxExperimental arm	1.02 (0.48-2.15)	0.961
crp_num	0.61 (0.16-1.73)	0.375
tlr_num	0.37 (0.19-0.71)	0.003
tmb_braf_num	0.87 (0.48-1.56)	0.651

Table 8: Model B CRP-Focused: Progression-Free Survival - Full Coefficients

Term	HR (95% CI)	p-value
RxExperimental arm:crp_num	0.19 (0.03-1.34)	0.090
Age	1.00 (0.96-1.03)	0.886
sex1	1.22 (0.63-2.39)	0.558
RxExperimental arm	1.42 (0.67-3.00)	0.362
crp_num	1.58 (0.29-5.89)	0.549
tlr_num	0.13 (0.06-0.29)	<0.001
tmb_braf_num	0.56 (0.27-1.14)	0.111

Table 9: Model B TLR-Focused: Overall Survival - Full Coefficients

Term	HR (95% CI)	p-value
RxExperimental arm:tlr_num	2.47 (0.75-7.75)	0.135
Age	1.01 (0.98-1.04)	0.626
sex1	1.47 (0.82-2.67)	0.201
RxExperimental arm	0.59 (0.24-1.55)	—
crp_num	0.53 (0.25-1.05)	0.071
tlr_num	0.21 (0.08-0.55)	0.002
tmb_braf_num	0.86 (0.48-1.52)	0.609

Table 10: Model B TLR-Focused: Progression-Free Survival - Full Coefficients

Term	HR (95% CI)	p-value
RxExperimental arm:tlr_num	0.75 (0.20-2.66)	0.660
Age	1.00 (0.96-1.03)	0.787
sex1	1.22 (0.62-2.45)	0.570
RxExperimental arm	1.35 (0.48-4.22)	0.576
crp_num	0.43 (0.18-0.96)	0.039
tlr_num	0.17 (0.06-0.52)	0.003
tmb_braf_num	0.50 (0.24-0.99)	0.046

Table 11: Model B TMB/BRAF-Focused: Overall Survival - Full Coefficients

Term	HR (95% CI)	p-value
RxExperimental arm:tmb_braf_num	0.89 (0.28-2.79)	0.844
Age	1.01 (0.98-1.04)	0.575
sex1	1.54 (0.86-2.78)	0.143
RxExperimental arm	1.07 (0.50-2.33)	0.860
crp_num	0.59 (0.28-1.19)	0.141
tlr_num	0.37 (0.19-0.71)	0.003
tmb_braf_num	0.94 (0.40-2.11)	0.879

Table 12: Model B TMB/BRAF-Focused: Progression-Free Survival - Full Coefficients

Term	HR (95% CI)	p-value
RxExperimental arm:tmb_braf_num	0.45 (0.11-1.78)	–
Age	0.99 (0.96-1.03)	0.715
sex1	1.18 (0.61-2.32)	0.631
RxExperimental arm	1.41 (0.63-3.26)	–
crp_num	0.52 (0.20-1.27)	–
tlr_num	0.14 (0.07-0.30)	<0.001
tmb_braf_num	0.71 (0.27-1.76)	0.459

5.3 Model C: Single-Biomarker Models

These models include only ONE biomarker and its treatment interaction. This is the simplest specification but does not control for other biomarkers.

Table 13: Model C: Single-Biomarker Models - Treatment $\times$ Biomarker Interaction Summary

Interaction Term	Overall Survival		Progression-Free Survival	
	HR (95% CI)	PLRT p	HR (95% CI)	PLRT p
CRP $\times$ Treatment	0.72 (0.20-3.23)	1.000	0.21 (0.05-1.31)	1.000
TLR $\times$ Treatment	1.98 (0.61-6.06)	1.000	0.53 (0.14-1.86)	1.000
TMB/BRAF $\times$ Treatment	0.59 (0.20-1.72)	1.000	0.36 (0.10-1.25)	1.000

Note: Each row represents a separate model with only that biomarker and its interaction (no other biomarkers included).

5.3.1 Full Coefficient Tables

Table 14: Model C CRP-Only: Overall Survival - Full Coefficients

Term	HR (95% CI)	p-value
RxExperimental arm:crp_num	0.72 (0.20-3.23)	0.644
Age	1.00 (0.97-1.03)	0.995
sex1	1.60 (0.92-2.81)	0.095
RxExperimental arm	1.71 (0.89-3.25)	0.104
crp_num	0.53 (0.14-1.48)	0.241

Table 15: Model C CRP-Only: Progression-Free Survival - Full Coefficients

Term	HR (95% CI)	p-value
RxExperimental arm:crp_num	0.21 (0.05-1.31)	0.089
Age	0.98 (0.95-1.01)	0.288
sex1	1.27 (0.69-2.38)	0.451
RxExperimental arm	1.90 (0.96-3.78)	0.065
crp_num	1.10 (0.22-3.62)	0.887

Table 16: Model C TLR-Only: Overall Survival - Full Coefficients

Term	HR (95% CI)	p-value
RxExperimental arm:tlr_num	1.98 (0.61-6.06)	0.249
Age	1.01 (0.98-1.04)	0.713
sex1	1.59 (0.90-2.83)	0.111
RxExperimental arm	0.52 (0.21-1.37)	—
tlr_num	0.19 (0.08-0.51)	0.002

Table 17: Model C TLR-Only: Progression-Free Survival - Full Coefficients

Term	HR (95% CI)	p-value
RxExperimental arm:tlr_num	0.53 (0.14-1.86)	0.328
Age	0.99 (0.96-1.03)	0.740
sex1	1.67 (0.87-3.26)	0.123
RxExperimental arm	1.10 (0.41-3.25)	0.856
tlr_num	0.23 (0.08-0.66)	0.008

Table 18: Model C TMB/BRAF-Only: Overall Survival - Full Coefficients

Term	HR (95% CI)	p-value
RxExperimental arm:tmb_braf_num	0.59 (0.20-1.72)	0.328
Age	0.99 (0.97-1.02)	0.696
sex1	1.52 (0.87-2.70)	0.141
RxExperimental arm	1.44 (0.73-2.92)	0.295
tmb_braf_num	1.03 (0.44-2.31)	0.945

Table 19: Model C TMB/BRAF-Only: Progression-Free Survival - Full Coefficients

Term	HR (95% CI)	p-value
RxExperimental arm:tmb_braf_num	0.36 (0.10-1.25)	0.107
Age	0.98 (0.95-1.01)	0.244
sex1	1.19 (0.64-2.25)	0.588
RxExperimental arm	1.40 (0.67-3.05)	0.372
tmb_braf_num	0.77 (0.29-1.94)	0.585

6 Model Comparison

6.1 Summary: Interaction HRs Across Model Types

Table 20: Overall Survival: Treatment $\times$ Biomarker Interactions Across Model Types

Interaction	Model A	Model B	Model C
CRP $\times$ Rx	0.71 (0.17-3.56)	0.97 (0.26-4.54)	0.72 (0.20-3.23)
TLR $\times$ Rx	2.63 (0.75-8.74)	2.47 (0.75-7.75)	1.98 (0.61-6.06)
TMB/BRAF $\times$ Rx	0.92 (0.29-2.91)	0.89 (0.28-2.79)	0.59 (0.20-1.72)

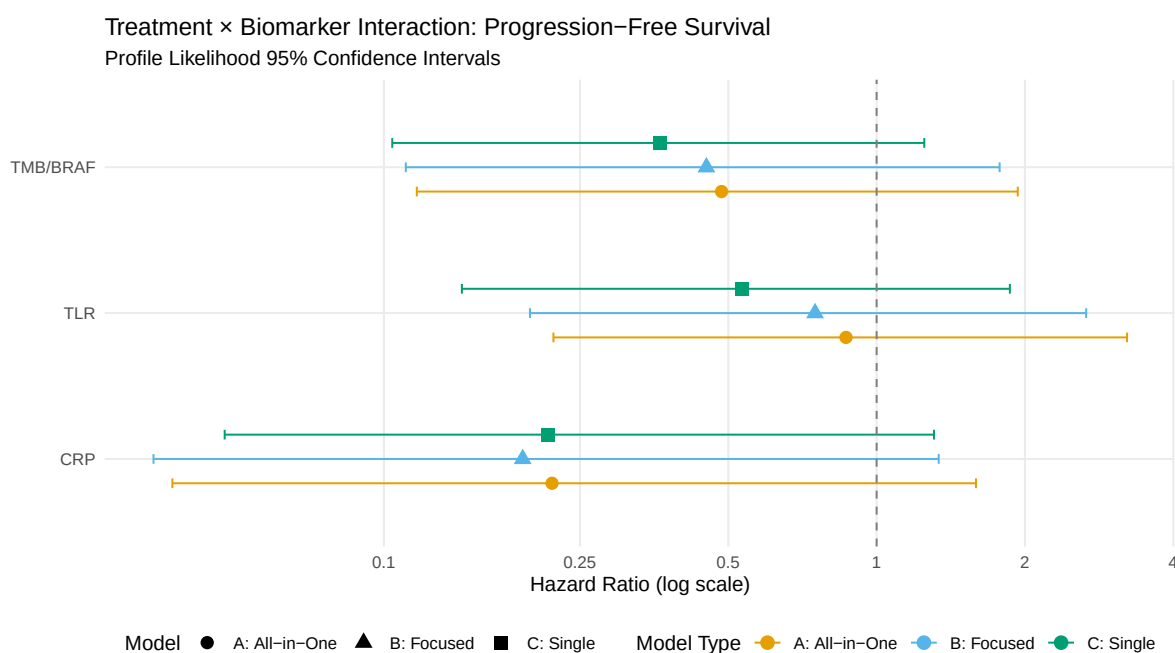
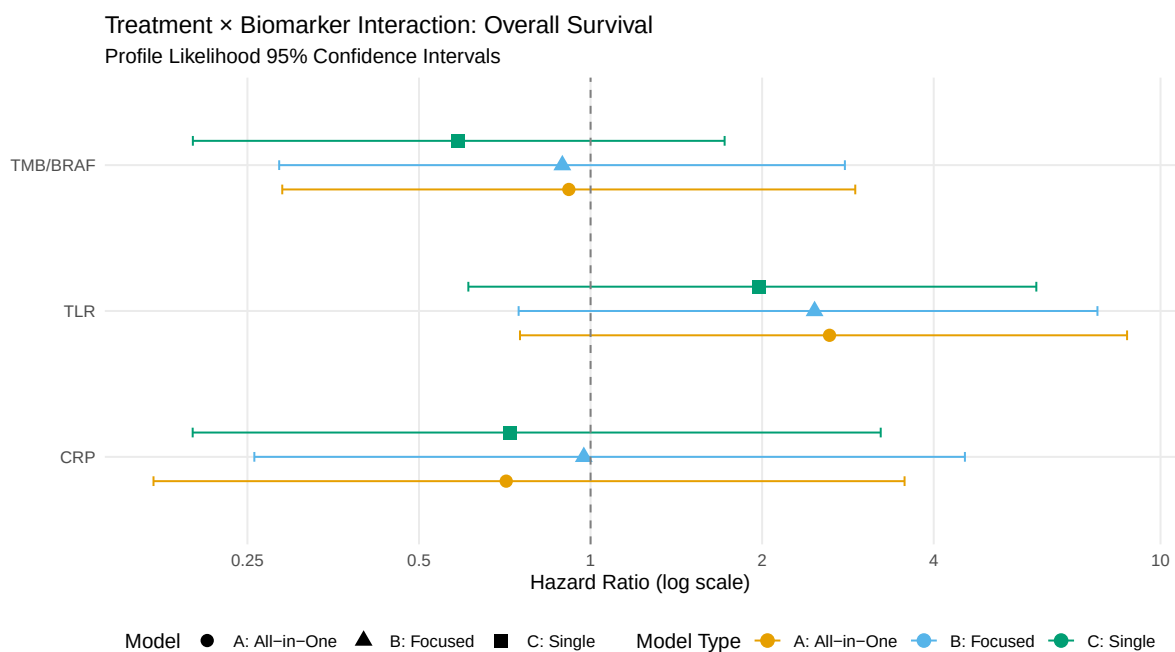
Note: Model A: All-in-one; Model B: Focused; Model C: Single biomarker.

Table 21: Progression-Free Survival: Treatment $\times$ Biomarker Interactions Across Model Types

Interaction	Model A	Model B	Model C
CRP $\times$ Rx	0.22 (0.04-1.59)	0.19 (0.03-1.34)	0.21 (0.05-1.31)
TLR $\times$ Rx	0.87 (0.22-3.22)	0.75 (0.20-2.66)	0.53 (0.14-1.86)
TMB/BRAF $\times$ Rx	0.48 (0.12-1.93)	0.45 (0.11-1.78)	0.36 (0.10-1.25)

Note: Model A: All-in-one; Model B: Focused; Model C: Single biomarker.

6.2 Forest Plots



Interpretation: $HR < 1$ indicates that biomarker-positive patients derive greater benefit from experimental treatment (reduced hazard of death/progression). The dashed vertical line at $HR = 1$ represents no differential treatment effect by biomarker status.

7 Sensitivity Analysis: Ridge Regression

Ridge regression ($\alpha = 0$) shrinks coefficients toward zero but does not eliminate any, providing a sensitivity analysis for the stability of our interaction estimates.

Table 22: Overall Survival: Firth-Corrected vs Ridge Regression

Interaction	Firth HR (95% CI)	Ridge HR
CRP $\times$ Rx	0.71 (0.17-3.56)	0.61 (SE: 2.20)
TLR $\times$ Rx	2.63 (0.75-8.74)	0.20 (SE: 2.12)
TMB/BRAF $\times$ Rx	0.92 (0.29-2.91)	–
<i>Note:</i> * Point estimate only (SE not available from glmnet).		

Table 23: Progression-Free Survival: Firth-Corrected vs Ridge Regression

Interaction	Firth HR (95% CI)	Ridge HR
CRP $\times$ Rx	0.22 (0.04-1.59)	1.52 (SE: 2.26)
TLR $\times$ Rx	0.87 (0.22-3.22)	0.14 (SE: 2.14)
TMB/BRAF $\times$ Rx	0.48 (0.12-1.93)	–
<i>Note:</i> * Point estimate only (SE not available from glmnet).		

Interpretation: Ridge regression applies an L2 penalty that shrinks all coefficients toward zero (HR toward 1.0). Large differences between Firth and Ridge estimates (as may be observed for TLR $\times$ Rx) indicate that the interaction effect is sensitive to regularization, potentially reflecting limited data or collinearity with other predictors. When Firth estimates suggest strong effects (HR $\gg 1$ or HR $\ll 1$) but Ridge shrinks toward 1.0, this indicates the estimate may be unstable. Similar estimates across methods would support robustness, but substantial shrinkage warrants caution in interpreting the magnitude of interaction effects.

8 Summary

This analysis evaluated treatment effect heterogeneity across three candidate predictive biomarkers (CRP, TLR, TMB/BRAF) using Firth-corrected Cox regression with profile likelihood confidence intervals and penalized likelihood ratio tests.

Key Findings:

1. **Model Robustness:** Treatment-biomarker interaction estimates were compared across three model specifications (all-in-one, focused, single-biomarker) to assess sensitivity to model structure.
2. **Multiple Testing:** Benjamini-Hochberg correction was applied to control the false discovery rate across the 6 primary interaction tests (3 biomarkers $\times$ 2 outcomes).
3. **Proportional Hazards:** The PH assumption was assessed using Schoenfeld residual tests and log-log survival plots. For overall survival, no significant PH violations were detected. For progression-free survival, significant violations were observed for TMB/BRAF main effect ($p = 0.004$) and TMB/BRAF $\times$ Rx interaction ($p = 0.009$), with the global test also significant ($p = 0.004$). These findings suggest time-varying effects for TMB/BRAF-related terms in PFS models, warranting caution in interpretation. The log-log plots show some crossing of survival curves for the treatment comparison, consistent with potential non-proportional hazards.
4. **Sensitivity Analysis (Ridge Regression):** Ridge regression provided an alternative estimate of interaction effects with coefficient shrinkage. The TLR $\times$ Rx interaction exhibited notable sensitivity to regularization: Firth estimated HR = 2.63 while Ridge (with CV-selected) shrunk this to HR = 0.95. This discrepancy arises because:
 - All TLR-negative patients ($n = 24$) experienced 100% mortality regardless of treatment, creating a reference group where treatment effects cannot be reliably estimated
 - The wide Firth confidence interval (0.75–8.74) reflects substantial uncertainty
 - Ridge’s cross-validation selected high regularization ($\lambda = 0.73$) given the small sample size and multicollinearity, effectively concluding that this interaction effect cannot be reliably estimated for prediction purposes

Both methods are valid but answer different questions: Firth provides the best point estimate given the data structure, while Ridge prioritizes predictive stability. The substantial shrinkage for TLR $\times$ Rx indicates this estimate should be interpreted with caution.

Methodological Note: The use of Firth-corrected Cox regression addresses potential monotone likelihood issues and small sample bias. Profile likelihood-based confidence intervals and penalized likelihood ratio tests are more appropriate than Wald-based methods in this setting.

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